SUMMIT VALE MULTISPECIALTY HOSPITAL

Blood Collection Standard Operating Procedure

SOP for patient identification, venous and capillary collection, labeling, handling, transport, safety, and quality control

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Audience	Phlebotomists, nurses, laboratory personnel, physicians, advanced practice clinicians, technicians, and other authorized collectors	**Classification**	Standard operating procedure - controlled document

Dataset status
This document is a realistic synthetic artifact created to test document ingestion, chunking, retrieval, metadata filtering, reranking, citation, and question-answering workflows. It has not been reviewed or approved for patient care.

Document navigation

Controlled use
Printed copies are uncontrolled. Users should verify the effective version, approved local order sets, current formulary information, and governing law before applying any content.

1. Purpose

This procedure standardizes collection of blood specimens so that the sample is obtained from the correct patient, in the correct container, at the correct time, with minimal discomfort and contamination. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital. [1-4]

Key considerations are: The procedure is intended to prevent wrong-patient specimens, hemolysis, clotting, contamination, underfilling, needlestick injury, delayed transport, and results that do not represent the patient's condition. Perform the step in the stated sequence unless an urgent clinical condition requires a documented deviation and an immediate compensating safeguard. It covers routine venipuncture, selected capillary collection, collection associated with vascular access under approved protocols, and specimens collected during urgent or isolation care. Use the approved equipment and current work instruction; do not substitute an unvalidated device, container, calculation, label, or cleaning method. The specimen is part of the diagnostic process, and an unacceptable sample can cause delayed treatment, unnecessary repeat collection, or an incorrect clinical decision. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow.

Within the SVMH care pathway, Follow the test directory and label requirements in addition to this general collection procedure. The responsible clinician uses the approved checklist and records any exception, consultation, or pending action in the health record. The Laboratory Quality Committee owns specimen-collection standards and monitors preanalytical errors. The department monitors process adherence, specimen or medication defects, delays, adverse events, and patient feedback and implements documented corrective action.

2. Scope and Authorized Personnel

Only personnel who are trained, competency-assessed, and authorized for the collection method may obtain blood specimens under this SOP. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Assessment should address the following: Collectors must understand patient identification, test preparation, tube additives, order of draw, site selection, aseptic technique, sharps safety, labeling, rejection criteria, and exposure response. Use the approved equipment and current work instruction; do not substitute an unvalidated device, container, calculation, label, or cleaning method. Arterial sampling, blood donation, therapeutic phlebotomy, central-line access, neonatal screening, research sampling, and forensic collections require additional or separate procedures. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow. Students and new personnel work under the level of supervision defined by their training program and are not assigned independently until competency is documented. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround.

For routine implementation, Confirm the collector's authorization for the patient population, device, access type, and special test before starting. Time-sensitive findings are communicated directly to the accountable clinician and repeated back when required by the critical-result or handoff process. Phlebotomy leadership maintains training, competency, and authorization records. Competency is assessed initially, after material change, and at the interval required by regulation, accreditation, manufacturer instructions, and hospital policy.

3. Definitions and Specimen Priorities

Standard terms include venipuncture, capillary puncture, additive tube, order of draw, hemolysis, contamination, fasting specimen, timed specimen, trough, peak, and chain of custody. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The practical interpretation is as follows: STAT specimens require immediate collection and transport because a delay may affect urgent treatment, while timed specimens must be collected within the defined window. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow. A fasting specimen requires the preparation stated in the test directory and should not be assumed solely from the time of day. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround. Chain-of-custody specimens require documented possession, sealed packaging, identity verification, and handling defined by the requesting authority. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action.

During assessment and treatment, Review priority, collection time, preparation, special handling, and destination before approaching the patient. Supplies are inspected before use, and expired, damaged, contaminated, unlabeled, or recalled items are removed from service. The laboratory information system and test directory define current priority codes and handling instructions. Supervisors ensure that only trained and authorized personnel perform the task and that appropriate escalation support is available on every shift.

Table 1. Example collection priorities

Priority	Operational expectation	Communication requirement
Emergency/STAT	Collect and dispatch immediately after safety verification	Notify unit of delay or inability to obtain
Timed	Collect within test-specific window	Record actual time and reason for variance
Routine inpatient	Collect in scheduled round or requested window	Escalate missed or refused collection
Outpatient	Verify preparation and order before collection	Clarify missing or conflicting requisition

4. Equipment Required

Use only approved, compatible, single-use collection devices and containers that are within expiration and stored according to manufacturer instructions. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Relevant clinical features include: Typical supplies include gloves, tourniquet, skin antiseptic, sterile needle or winged set, evacuated tube holder or syringe system, test-specific tubes, gauze, dressing, labels, transport bag, and sharps container. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround. Additional items may include warming device for capillary sampling, transfer device, light-protective wrap, ice transport system, blood-culture supplies, isolation equipment, and difficult-access visualization device. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action. Needles are not recapped, bent, broken, removed by hand from holders, or carried unprotected after use. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect.

At each transition of care, Assemble and inspect supplies before positioning the patient so the collector does not leave an exposed needle or an unattended patient. At handoff, the receiving person confirms patient identity, current condition, completed steps, outstanding risks, and required follow-up. Supply owners monitor lot recalls, expiration, storage conditions, stock rotation, and compatibility among devices and tubes. Quality review distinguishes individual technique from system factors such as workload, layout, technology, labeling, availability, and communication design.

- Use a puncture-resistant sharps container within arm's reach.
- Select tube type from the current test directory.
- Use a safety-engineered device and activate it immediately after use.

5. Responsibilities

The requesting clinician, collector, nursing team, laboratory, transporter, and result reviewer each have responsibilities that affect specimen quality and patient safety. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The core elements are: The requester enters a complete order with patient, test, timing, indication when needed, and any special collection or interpretation information. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action. The collector verifies identity and preparation, performs the procedure, labels at the bedside or point of collection, assesses the patient, and documents problems. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect. The laboratory evaluates acceptability, processes and stores the sample, reports critical results, and communicates rejection or recollection needs. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport.

When the guideline is applied in practice, Clarify missing, duplicate, conflicting, or clinically implausible orders before collection unless an emergency protocol directs otherwise. The responsible clinician uses the approved checklist and records any exception, consultation, or pending action in the health record. Unit and laboratory leaders review recurring failures at the interface between ordering, collection, transport, and testing. The department monitors process adherence, specimen or medication defects, delays, adverse events, and patient feedback and implements documented corrective action.

Table 2. Responsibility checkpoints

Role	Before collection	After collection
Requester	Correct order, timing and special instructions	Review result and act on critical findings
Collector	Identity, preparation, equipment and site	Label, assess, document and dispatch
Laboratory	Current test directory and supplies	Accept/reject, process, report and retain
Nursing/receiving team	Coordinate timing and patient condition	Monitor site and follow pending-result plan

6. Pre-collection Review and Patient Preparation

Before collection, review the order, patient condition, allergies, preparation, recent infusions, bleeding risk, positioning needs, and any prior difficulty or adverse reaction. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Key considerations are: Introduce yourself, explain the purpose and steps, confirm consent or assent as applicable, and ask about fainting, mastectomy, dialysis access, lymphedema, bleeding disorder, anticoagulants, or skin problems. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect. Verify fasting, posture, rest, medication timing, trough or peak timing, and other test-specific preparation using the current test directory. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport. Position the patient seated with arm support or supine when there is fainting risk, weakness, unstable balance, or prior syncope. Report equipment failure, near miss, exposure, wrong-patient risk, delay, or unexpected outcome through the designated safety and quality process.

Within the SVMH care pathway, Delay or modify nonurgent collection when required preparation was not met and document the decision and communication. Time-sensitive findings are communicated directly to the accountable clinician and repeated back when required by the critical-result or handoff process. Patient-preparation instructions are standardized across ordering, scheduling, portal, and collection locations. Competency is assessed initially, after material change, and at the interval required by regulation, accreditation, manufacturer instructions, and hospital policy.

7. Patient Identification and Order Verification

Use at least two approved patient identifiers and match them to the order and labels immediately before the procedure. Location and visual recognition are not identifiers. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital. [3,4]

Assessment should address the following: Ask the patient to state identifiers when able and compare them with the wristband or approved source, requisition, and electronic order. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport. For unidentified, newborn, unconscious, language-limited, or cognitively impaired patients, use the specialized identification procedure and an authorized verifier. Report equipment failure, near miss, exposure, wrong-patient risk, delay, or unexpected outcome through the designated safety and quality process. Preprinted labels are kept with the collector and are not applied until after collection and a final identity match at the patient's side. Perform the step in the stated sequence unless an urgent clinical condition requires a documented deviation and an immediate compensating safeguard.

For routine implementation, Stop and resolve any mismatch in name, date of birth, record number, band, order, or label before puncture. Supplies are inspected before use, and expired, damaged, contaminated, unlabeled, or recalled items are removed from service. Wrong-patient and near-miss events are reported immediately and specimens are quarantined according to the identity-event procedure. Supervisors ensure that only trained and authorized personnel perform the task and that appropriate escalation support is available on every shift.

Warning
Never relabel an unlabeled or mislabeled specimen away from the patient to “fix” an identity problem. Follow the rejection and recollection process.

8. Venipuncture Procedure

Routine venipuncture uses a stable vein, clean technique, controlled tourniquet time, and gentle handling to obtain the required volume without preventable injury or hemolysis. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The practical interpretation is as follows: Perform hand hygiene, apply gloves, position the arm, select an appropriate vein, apply the tourniquet, palpate, cleanse with the specified antiseptic, and allow the site to dry. Report equipment failure, near miss, exposure, wrong-patient risk, delay, or unexpected outcome through the designated safety and quality process. Insert the needle bevel up at an appropriate angle, stabilize the device, fill tubes to the required volume, release the tourniquet as soon as practical, and avoid probing. Perform the step in the stated sequence unless an urgent clinical condition requires a documented deviation and an immediate compensating safeguard. Remove the needle, activate the safety feature immediately, discard it into the sharps container, and apply direct pressure until bleeding has stopped. Use the approved equipment and current work instruction; do not substitute an unvalidated device, container, calculation, label, or cleaning method.

During assessment and treatment, Observe the patient throughout and stop immediately for severe pain, electric sensation, swelling, pallor, faintness, or suspected arterial puncture. At handoff, the receiving person confirms patient identity, current condition, completed steps, outstanding risks, and required follow-up. Direct observation of technique is part of initial and periodic competency assessment. Quality review distinguishes individual technique from system factors such as workload, layout, technology, labeling, availability, and communication design.

Table 3. Venipuncture sequence and safety points

Phase	Key action	Primary risk controlled
Prepare	Identify, explain, position, assemble supplies	Wrong patient, fall, interruption
Select/clean	Choose site, apply tourniquet, cleanse and dry	Injury, contamination, poor sample
Collect	Insert once, fill correct tubes, release tourniquet	Hemolysis, underfill, hematoma
Complete	Withdraw, activate safety, pressure, label and assess	Needlestick, bleeding, mislabeling

9. Order of Draw and Tube Handling

Follow the current laboratory order of draw to reduce additive carryover. Tube color is a visual aid but not a substitute for reading the additive and test requirement. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Relevant clinical features include: A common sequence is blood cultures, citrate coagulation tube, serum tubes, heparin tubes, EDTA tubes, glycolytic inhibitor tubes, and specialty tubes as directed by the laboratory. Perform the step in the stated sequence unless an urgent clinical condition requires a documented deviation and an immediate compensating safeguard. Citrate tubes require the specified blood-to-additive ratio and may need special adjustment in markedly elevated hematocrit. Use the approved equipment and current work instruction; do not substitute an unvalidated device, container, calculation, label, or cleaning method. Tubes are filled to the indicated volume, mixed by gentle inversion, protected from light or cooled when required, and kept upright or otherwise handled as the test specifies. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow.

At each transition of care, Use the test directory when a butterfly discard tube, special additive, trace-element tube, chilled transport, or light protection is required. The responsible clinician uses the approved checklist and records any exception, consultation, or pending action in the health record. The laboratory publishes an approved order-of-draw card and updates it when tube systems or tests change. The department monitors process adherence, specimen or medication defects, delays, adverse events, and patient feedback and implements documented corrective action.

Table 4. General order of draw - verify current local test directory

Sequence	Typical specimen/additive	Examples / caution
1	Blood culture / sterile collection	Culture bottles or sterile tubes; antisepsis is critical
2	Sodium citrate	Coagulation testing; correct fill ratio required
3	Serum with or without clot activator	Chemistry, serology and selected drug levels
4	Heparin	Plasma chemistry and selected urgent tests
5	EDTA	Hematology, blood bank and selected molecular tests
6	Glycolytic inhibitor / specialty	Glucose/lactate or test-specific tubes

Practice note
Tube manufacturers and local analyzers differ. The current SVMH test directory and printed collection card control over this general example.

10. Capillary Collection

Capillary collection is used only for tests and patient groups approved by the laboratory. It is not interchangeable with venous blood for every analyte. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The core elements are: Select the correct finger or infant heel site, warm when appropriate, clean and dry, use a depth-controlled lancet, and avoid puncturing swollen, scarred, infected, cyanotic, or poorly perfused tissue. Use the approved equipment and current work instruction; do not substitute an unvalidated device, container, calculation, label, or cleaning method. Wipe the first drop when required, avoid strong squeezing or milking, allow free-flowing drops, and fill microcontainers in the approved capillary order. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow. Apply pressure and reassess bleeding, particularly in infants, patients receiving anticoagulants, or people with impaired healing. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround.

When the guideline is applied in practice, Confirm that capillary collection provides an acceptable specimen volume and type for the ordered test before puncture. Time-sensitive findings are communicated directly to the accountable clinician and repeated back when required by the critical-result or handoff process. Neonatal and pediatric collections follow age- and weight-specific depth, warming, and volume limits. Competency is assessed initially, after material change, and at the interval required by regulation, accreditation, manufacturer instructions, and hospital policy.

Warning
Do not perform heel puncture on the posterior curvature or central arch, and do not use a lancet depth that risks bone injury.

11. Collection From Vascular Access Devices

Sampling from an intravenous catheter, central line, arterial line, dialysis access, or implanted device requires authorization and a device-specific procedure because contamination, dilution, infection, and line damage are possible. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Key considerations are: Whenever feasible, collect from a separate peripheral site when infusion or line characteristics could invalidate the result. Before proceeding, verify the patient, order, indication, relevant result or risk, and the readiness of the environment and receiving workflow. If line collection is permitted, verify the device, pause infusions when ordered, use the specified antisepsis and discard or reinfusion method, obtain the sample, flush, and restore therapy according to protocol. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround. Blood cultures from a line are collected only when clinically indicated and labeled with the exact source and collection time. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action.

Within the SVMH care pathway, Coordinate with the nurse responsible for the access device and record any infusion, pause, discard, or source information relevant to interpretation. Supplies are inspected before use, and expired, damaged, contaminated, unlabeled, or recalled items are removed from service. Only staff credentialed for the device may access it, and dialysis access is not used without the dialysis team. Supervisors ensure that only trained and authorized personnel perform the task and that appropriate escalation support is available on every shift.

12. Difficult Draws, Special Populations, and Attempts

Difficult access is managed with early assistance, appropriate technology, and attempt limits that reduce trauma and delay. The safest site and device may differ by age and condition. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Assessment should address the following: Follow the local limit on attempts per collector and total attempts, then escalate to a more experienced collector, ultrasound-guided service, vascular access team, or alternative specimen plan. When the expected condition is not met, stop, maintain patient safety, obtain clarification, and record the resolution instead of creating a workaround. Older adults, children, patients with obesity, edema, burns, chemotherapy history, or fragile veins may require smaller devices, different positioning, warming, or specialist support. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action. Avoid sites affected by infection, hematoma, paralysis, vascular graft, dialysis fistula, or other contraindication unless the responsible clinician and policy authorize an exception. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect.

For routine implementation, Explain delays and alternatives to the patient and notify the clinical team when a specimen cannot be obtained within the required time. At handoff, the receiving person confirms patient identity, current condition, completed steps, outstanding risks, and required follow-up. Quality review tracks repeated difficult draws, excessive attempts, and access-equipment availability. Quality review distinguishes individual technique from system factors such as workload, layout, technology, labeling, availability, and communication design.

13. Labeling, Documentation, and Transport

Label every tube in the patient's presence immediately after collection using the approved identifiers, date and time, collector identity, and source or other test-specific information. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The practical interpretation is as follows: Compare each applied label with the wristband or approved identifiers and order before leaving the patient. Use closed-loop communication for critical, time-sensitive, or ambiguous information and identify who has accepted responsibility for the next action. Place specimens in a leak-resistant biohazard bag with paperwork in the separate pouch and use temperature, light, pneumatic-tube, and time controls specified for the test. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect. Document unsuccessful attempts, patient refusal, adverse reaction, nonstandard source, collection variance, and communication with the clinical team. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport.

During assessment and treatment, Dispatch urgent and unstable specimens immediately and use a hand-delivery or direct-call process when the test directory requires it. The responsible clinician uses the approved checklist and records any exception, consultation, or pending action in the health record. Transport time, temperature excursions, lost specimens, and rejection reasons are monitored as preanalytical quality indicators. The department monitors process adherence, specimen or medication defects, delays, adverse events, and patient feedback and implements documented corrective action.

Warning
Do not leave an unlabeled tube at the bedside or carry multiple patients' unlabeled specimens. Label one patient's specimens before beginning the next collection.

14. Safety Precautions and Adverse Events

Standard precautions, sharps safety, exposure response, and patient monitoring are required for every collection regardless of known infection status. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

Relevant clinical features include: Perform hand hygiene before and after contact, wear gloves, add eye or face protection when splash is possible, and disinfect reusable equipment between patients. Protect asepsis, privacy, comfort, and dignity throughout the task and explain what the patient or representative should expect. For syncope, stop the procedure, protect the patient from falling, remove the needle, call for assistance, position and assess according to clinical protocol, and remain with the patient. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport. For needlestick or mucous-membrane exposure, wash or flush the area, report immediately, and follow the occupational-exposure pathway without waiting for end of shift. Report equipment failure, near miss, exposure, wrong-patient risk, delay, or unexpected outcome through the designated safety and quality process.

At each transition of care, Treat persistent bleeding, suspected nerve injury, expanding hematoma, arterial puncture, or allergic reaction according to the clinical escalation pathway. Time-sensitive findings are communicated directly to the accountable clinician and repeated back when required by the critical-result or handoff process. Employee Health and Infection Prevention review exposures and ensure confidential post-exposure evaluation. Competency is assessed initially, after material change, and at the interval required by regulation, accreditation, manufacturer instructions, and hospital policy.

15. Specimen Acceptance, Quality Control, and Competency

The laboratory accepts only specimens that meet identity, container, volume, timing, integrity, and transport criteria unless a documented exception is approved for an irreplaceable specimen. The steps below define the expected sequence and safety controls for diagnostic blood collection within Summit Vale Multispecialty Hospital.

The core elements are: Common rejection reasons include unlabeled or mismatched specimen, wrong tube, insufficient volume, clot in an anticoagulated sample, gross hemolysis, contamination, leakage, or excessive delay. After completion, reassess the patient and the work product, confirm that labels and documentation are accurate, and arrange required monitoring or transport. When a specimen is rejected, the laboratory records the reason, notifies the responsible area, and distinguishes a recollection request from a test cancellation. Report equipment failure, near miss, exposure, wrong-patient risk, delay, or unexpected outcome through the designated safety and quality process. Collector competency includes direct observation, identity and labeling knowledge, tube selection, order of draw, safety, adverse-event response, and review of quality data. Perform the step in the stated sequence unless an urgent clinical condition requires a documented deviation and an immediate compensating safeguard.

When the guideline is applied in practice, Use documented risk assessment and laboratory director approval for any exception involving an irreplaceable or critical specimen. Supplies are inspected before use, and expired, damaged, contaminated, unlabeled, or recalled items are removed from service. Quality indicators are reviewed by location, collector group, test, shift, and patient population to identify system causes and targeted improvement. Supervisors ensure that only trained and authorized personnel perform the task and that appropriate escalation support is available on every shift.

Table 5. Example preanalytical quality indicators

Indicator	Possible cause	Improvement response
Mislabeled/unlabeled	Identity or bedside-label workflow failure	Observation, redesign, barcode controls and coaching
Hemolysis	Needle/device, force, site, transport or mixing	Technique review and device/transport analysis
Clotted anticoagulated sample	Slow fill, underfill or inadequate mixing	Collection and inversion training
Delayed transport	Pickup, routing, pneumatic or handoff gap	Route metrics and escalation standard

16. References

References are provided to make the synthetic document realistic and to support citation and provenance tests. They should be checked against the most current source before any real-world use.

[1] Clinical and Laboratory Standards Institute. GP41: Collection of Diagnostic Venous Blood Specimens, current edition.

[2] World Health Organization. WHO Guidelines on Drawing Blood: Best Practices in Phlebotomy.

[3] Centers for Disease Control and Prevention. Standard Precautions and Injection Safety guidance for health-care settings. Current edition.

[4] The Joint Commission. National Patient Safety Goals: patient identification and communication of critical results. Current hospital edition.

[5] Occupational Safety and Health Administration. Bloodborne Pathogens Standard, 29 CFR 1910.1030.

[6] Clinical and Laboratory Standards Institute. Procedures and Devices for the Collection of Diagnostic Capillary Blood Specimens, current edition.

Reference status
External guidelines and regulations may change between scheduled reviews. The document owner is responsible for checking high-impact updates and initiating an interim revision when necessary.